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CUSTOMER :	IDAMAN PHARMA MANUFACTURING SDN BHD
DESCRIPTION :	Cloxacilla Capsule 250mg
MEASUREMENT :	W:143MM X H: 143MM
MATERIAL :	SMPO 050

CLOUR : BLACK

FINISHING : FOLDING

MATERIAL CODE:

Lt-603.10

Attn : MS. SITI
From : MALA
Please verify & confirm that this draft is the final copy. Any subsequent changes of the design and content of the end product will not be borne by THUNDER PRINT SDN. BHD.
Customer Signature & Date

27.11.2020

IDAMAN PHARMA MANUFACTURING

CLOXACILLA CAPSULE 250 MG

COMPOSITION

Each capsule contains *CLOXACILLIN SODIUM* equivalent to 250 mg of *CLOXACILLIN*.

DESCRIPTION

A white to slightly yellow powder filled into Scarlet/Ivory (Red/Yellow).
Size: No. 2 Capsules.
Free from dust and powder.

PHARMACODYNAMICS

Cloxacillin sodium is an antibiotic belonging to the semi-synthetic penicillin family. Cloxacillin is a bactericidal antibiotic which inhibits bacterial cell wall synthesis, probably by acylation of membrane-bound transpeptidase enzymes. This prevents cross-linkage of peptidoglycan chains which is necessary for bacterial cell wall strength and rigidity. Also, cell division and growth are inhibited and lysis of susceptible bacteria frequently occurs. Rapidly dividing bacteria are most susceptible to penicillin group of antibiotics. Cloxacillin is resistant to staphylococcal penicillinase. It has an antibacterial spectrum similar to that of methicillin.

PHARMACOKINETICS

Cloxacillin sodium is incompletely absorbed from the gastrointestinal tract after oral administration, and absorption is further reduced by the presence of food in the stomach. After an oral dose of 500mg, a peak plasma concentration of 7 to 15 microgram per ml is obtained in fasting subjects in 1 to 2 hours. Doubling the dose can double the plasma concentration. About 94% of cloxacillin in the circulation are bound to plasma proteins. Cloxacillin has been reported to have a plasma half-life of approximately 0.5 to 1 hour. The half-life is prolonged in neonates.

Cloxacillin diffuses across the placenta into the foetal circulation and is excreted in human breast milk. There is little diffusion into the CFS except when the meninges are inflamed. Therapeutic concentrations can be achieved in pleural and synovial fluids and in bone. Cloxacillin is metabolized to a limited extent, and the unchanged drug and metabolites are excreted in the urine by glomerular filtration and renal tubular secretion. About 35% of an oral dose is excreted in the urine and up to 10% in the bile. Cloxacillin is not removed by haemodialysis.

Plasma concentrations are enhanced by probenecid. Reduced concentrations in patient with cystic fibrosis have been attributed to both enhanced tubular secretion and enhanced non-renal clearance of cloxacillin.

INDICATIONS

Cloxacilla, is used for infections due to Staphylococci resistant to benzylpenicillin including infections of the skin and soft tissues, bones and joints, respiratory tract and urinary tract; otitis media, endocarditis,

septicaemia and meningitis. It is also used for mixed Streptococcal and Staphylococcal infection when the Staphylococci are penicillin-resistant.

DOSAGE AND ADMINISTRATION

For adults: 250 - 500 mg every 6 hours daily
For children: 50 - 100 mg per kg body weight daily in divided doses every 6 hours.

The prescribed course should be completed. In severe infections, the doses may be safely increased. A daily dose of 6 g should not be exceeded. Reduced dosage is required in patients with impaired renal function.

INSTRUCTION FOR USE

Should be taken on an empty stomach. Take 1 hour before or 2 hours after meals.

ROUTE OF ADMINISTRATION

Oral

CONTRAINDICATIONS

Contraindicate in patient with known hypersensitivity to cloxacillin or history of allergy to penicillin group of drugs.

ADVERSE EFFECTS

As with other penicillins, adverse effects are usually mild and transitory in nature. This may include skin rashes, nausea, vomiting and diarrhea, fever, joint pains and angioedema.

Others include:

Hypersensitivity Reactions:

Eosinophilia, pruritus, serum sickness-like reaction with fever, chills, myalgia. Anaphylaxis and allergic reactions (rash, urticaria) including wheezing and sneezing, have occasionally been encountered.

Eosinophilia, with or without overt allergic manifestations, has been noted in some patients during therapy.

Haematologic Effects:

Haemolytic anaemia, transient neutropenia, leukopenia, granulocytopenia, thrombocytopenia, agranulocytosis.

Renal Effects:

Acute interstitial nephritis, azotemia.

Hepatic Effects:

Intrahepatic cholestasis.

Mildly elevated serum glutamic oxaloacetic transaminase (SGOT) levels (less than 100 units) have been reported in a few patients for pre-therapeutic determinations were not made.

Gastrointestinal Effects:

Gastrointestinal disturbances, such as nausea, vomiting, epigastric discomfort, flatulence and loose stools have been noted in some patients.

WARNINGS AND PRECAUTIONS

Serious and occasionally fatal hypersensitivity reactions (including anaphylactoid and severe cutaneous adverse reactions) have been reported in patients receiving therapy with beta-lactams. Before initiating therapy with Cloxacilla capsule, careful inquiry should be made concerning previous hypersensitivity reactions to penicillins, cephalosporins, carbapenems or other beta-lactams agents. If an allergic reaction occurs, Cloxacilla capsule must be discontinued immediately and appropriate alternative therapy instituted.

Candidiasis and other superinfections may occur, especially in debilitated and malnourished patients, or those with low resistance to infection due to corticosteroids, immunosuppressive agents or irradiation. If superinfection occurs, institute appropriate measures.

During long-term and high dose therapy, renal, hepatic and hematopoietic functions should be checked periodically.

Experience in premature and newborn infants is limited. Cautious administration of the drug to such patients and frequent evaluation of organ system function is recommended.

The passage of any penicillin from blood into brain is facilitated by inflamed meninges and during cardiopulmonary bypass. In the presence of such factors, particularly in renal failure when high serum concentrations can be attained, central nervous system adverse effects including myoclonia, convulsive seizures and depressed consciousness can be expected. Although this complication has not been reported with cloxacillin, it should be anticipated.

INTERACTION WITH OTHERS MEDICAMENTS

Clinically significant reductions in renal clearance have been noted for amoxicillin, nafcillin, and ticarcillin. It may also interact with bacteriostatic antibacterials such as chloramphenicol and tetracyclines.

Contraceptives, oestrogen, containing oral

Concurrent used with cloxacillin may decrease the effectiveness of oral contraceptives because of simulations of oestrogen metabolism or reduction in enterohepatic circulation of oestrogen, resulting in menstrual irregularities, intermenstrual bleeding unplanned pregnancies.

Probenecid

Probenecid prolongs the half-life by competing with it for renal tubular secretion and may be used therapeutically for this purpose.

PREGNANCY AND LACTATION

Cloxacilla Capsule should be used with caution in neonates, pregnancy and lactation.

OVERDOSE AND TREATMENT

When penicillin reaches a certain (as yet undetermined) concentration in the cerebrospinal fluid, neurotoxic symptoms may occur consisting of myoclonia, convulsive seizures and depressed consciousness. Unless administration of the drug is stopped or its dosage reduced, the syndrome may progress to coma and death. Penicillin does not normally

cross the blood-brain barrier to any substantial extent, but when massive doses are used (several grams a day) in the presence of inflamed meninges and/or impaired renal function, the drug may cause the above-mentioned toxic reactions. No antidote is required. Stop administration temporarily and promote excretion (dialysis, etc.). Toxic serum levels and the lethal serum level of cloxacillin in man are not known.

Overdosage may cause gastrointestinal tract (GIT) disturbances, reversible impairment of liver function and diarrhoea. Treatments include stopping antibiotic treatment and treating GIT distress by giving milk by every 3 hours alternating with bismuth subcarbonate 5g every 3 hours.

Problems of overdosage are unlikely to be encountered. Gross overdosage will produce very high urinary concentration, more so after parenteral administration. Problems are unlikely if adequate fluid intake and urinary output are maintained; however, crystalluria is a possibility. More specific measures may be necessary in patients with impaired renal function; the antibiotic is removed by haemodialysis.

STORAGE CONDITION

Store below 30°C, away from direct light.

SHELF LIFE

3 year from date of manufacture.

PRESENTATION

Malaysian Registration No.: MAL19890418AZ

Packed in blister of 10 capsules in carton box (50 x 10's)

Date of Revision: 27th November 2020

PRODUCT REGISTRATION HOLDER / MANUFACTURER: IDAMAN PHARMA MANUFACTURING SDN BHD (200401023395)

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